co-op.care

PARTNERSHIP PROPOSAL

The Care Commons

Blaine Warkentine → Cohesion Health

I want to join your team.

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THE SHORT VERSION

I'm a physician-technologist building a federation of worker-owned home care cooperatives — a Care Commons where care is a shared resource, not a commodity. I have the clinical team, the FHIR infrastructure partner, production-ready code (4 components, ~4,800 lines), and a clear revenue path. The platform now includes: a national federation model, a CareOS Health Intelligence dashboard powered by wearable data, a CareMCP server connecting Apple Watch / Oura / Garmin into clinical workflows, a three-account financial system (HSA/FSA + Lifestyle + Social Currency), Truemed-powered LMN automation for pre-tax eligibility, GivePulse-verified Time Bank credits, Unite Us closed-loop care referrals, and an Aging in Place insurance model that replaces reactive institutional care with proactive home-based intelligence. I'm not looking to hire Cohesion. I'm looking to join Cohesion's team as a client-developer. You build the production foundation. I build features on top of it. Your team reviews my code weekly, fixes what's broken, and teaches me to get better. In return: $5,000 cash, $15,000 equity, healthcare domain expertise from 20+ years, a 20,000-connection LinkedIn network, and an anchor client in regulated healthcare + MCP + social impact APIs.

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1. Who I Am + What I Bring to Cohesion

I'm not a typical client. I'm a healthcare operator who writes code, and I want to be part of your team.

1.1 My Background

Fellowship-trained orthopedic surgeon. Masters in Population Health. Built BrainLAB's orthopedic vertical from zero to market leader. Multiple exits. 5 patents. Two decades navigating the intersection of clinical care, technology, and healthcare business.

1.2 What I Offer Cohesion Beyond This Project

What I Bring	How It Helps Cohesion
Healthcare domain expertise (20+ years)	Healthcare SME for Cohesion's other clients. Clinical workflows, regulatory, billing, interoperability.
20,000+ LinkedIn connections	Active introductions to healthcare orgs, health systems, digital health companies.
FHIR / HL7 / CMS / SDOH technical knowledge	Advise on FHIR patterns, CMS compliance, Gravity Project SDOH integration, clinical data architecture.
MCP + health API expertise	CareMCP positions Cohesion at the intersection of MCP, wearables, and social impact APIs.
Case study: regulated healthcare + social impact	co-op.care showcases HIPAA, Medicare, FHIR, MCP, GivePulse, Unite Us, Truemed — in one platform.
Anchor client with expanding scope	Foundation sprint → CareMCP → FHIR → social impact APIs → insurance. Multi-year engagement.

2. The Care Commons: co-op.care

co-op.care is a federation of worker-owned home care cooperatives that treats care as a common-pool resource — a Care Commons — rather than a commodity. The Technologies LLC provides: technology platform, clinical governance framework, cooperative formation playbook, CareMCP health intelligence, and a social currency system that values informal care labor alongside professional care delivery.

Families pay $35/hour for in-home care. Caregivers earn $25–28/hour W-2 wages with equity ownership. Physician oversight makes it HSA/FSA eligible. CareMCP wearable monitoring catches problems before they become crises. And the Time Bank converts volunteer care into real economic value.

2.1 The Three-Account Financial Model

Every co-op.care member interacts with three financial accounts. This structure separates regulated healthcare spending, lifestyle wellness, and community care labor — each with its own funding source, compliance requirements, and API integrations.

Account	Funding Source	What It Covers	Key Integration
HSA/FSA (Pre-Tax)	Member pre-tax contributions via employer or individual plan	Medically necessary care with Letter of Medical Necessity: in-home caregiving, therapeutic services, DME, medication management	Truemed API: automated LMN issuance. Stripe MCC 8099. Josh's physician oversight.
Lifestyle Account (Post-Tax)	Cooperative membership dues, employer wellness stipends	Non-medical wellness: yoga memberships, nutrition coaching, ergonomic equipment, mental health apps, meal delivery	Mindbody API for class booking. Flex API for payment processing at partner merchants.
Social Currency (Earned)	Verified community care labor via GivePulse	Time Bank credits: companionship visits, transportation, meal prep, yard work, tech help. Redeemable for premium offsets, co-op services, or wellness account credits.	GivePulse API for verified hours. PIPs-style digital wallet. Credits mint at $15/hr base rate.

WHY THREE ACCOUNTS MATTER

Traditional home care is one-dimensional: family pays agency, agency pays caregiver (minus 40–60% extraction). co-op.care creates three distinct value streams: 1. HSA/FSA: Physician-supervised care is a qualified medical expense. Families save 28–36% via pre-tax dollars. Truemed automates the Letter of Medical Necessity so the tax benefit is frictionless. 2. Lifestyle: Wellness spending that doesn't require medical necessity — but still flows through the cooperative ecosystem, keeping members engaged and healthy. 3. Social Currency: The neighbor who checks on your mom, the

retired teacher who drives her to the doctor, the teenager who mows her lawn — this labor has real value. GivePulse verifies the hours. The cooperative mints credits. Credits reduce premiums, unlock services, or convert to wellness spending. This is the Time Bank made real.

2.2 Aging in Place vs. Traditional Long-Term Care

Traditional LTC insurance is a contingency product: it triggers only after someone has lost the ability to perform two or more Activities of Daily Living. It funds institutional care. The co-op.care Aging in Place (AIP) model inverts this logic: proactive monitoring, home modifications, and community support keep people at home and independent before a crisis.

Feature	Traditional LTC Insurance	co-op.care Aging in Place Model
Trigger	Loss of 2+ ADLs (reactive)	Proactive health markers from CareMCP wearables (preventative)
Primary Goal	Fund institutional care	Preserve independence at home
Key Coverage	Nursing homes, assisted living	Home mods, companionship, smart sensors, caregiver matching
Cost Model	High premiums, use-it-or-lose-it	Social currency offsets, HSA/FSA eligibility, reward-driven
Data Source	Periodic claims data	Continuous CareMCP biometrics + CII/CRI + daily care notes
Family Role	Visitor	Paid caregiver via CDPAP model with cooperative equity

2.3 The Caregiver Commons: Family as Worker-Owner

Family caregivers provide $600 billion in unpaid labor annually. co-op.care formalizes this labor through the cooperative membership structure. Following the Consumer Directed Personal Assistance Program (CDPAP) model, family members and friends can be hired as W-2 caregivers within the cooperative — earning wages, building equity, and receiving benefits.

Pathway	How It Works	Revenue Impact
Family Caregiver Onboarding	Family member completes 40-hour training. Josh certifies competency. Cooperative hires as W-2 at $25–28/hr.	Private pay from family's HSA/FSA (pre-tax). Family saves 28–36% vs. paying out-of-pocket for the same care.
Medicaid CDASS Integration	For Medicaid-eligible seniors, Colorado's Consumer Directed Attendant Support Services program allows family to be paid caregiver at $15–	Medicaid reimbursement flows through cooperative. Family member becomes worker-owner.

Pathway	How It Works	Revenue Impact
	22/hr.	
Caregiver Credit Tax Benefits	Federal Credit for Caring Act (proposed): $5,000 tax credit covering 30% of out-of-pocket caregiving expenses.	Stacks with HSA/FSA pre-tax benefit. Combined savings of 50% + for qualifying families.
Time Bank Neighbor Care	Non-family community members provide companionship, transport, meal prep. Hours verified via GivePulse. Credits earned.	No direct revenue but reduces professional care hours needed. Extends cooperative reach. Builds membership pipeline.

2.4 The Team

Role	Person	Details
Founder	Blaine Warkentine, MD MPH	Orthopedic surgeon. Healthcare technology. 5 patents, multiple exits.
Clinical Director	Josh Emdur, DO	BCH hospitalist since 2008. Former CMO SteadyMD. 50-state licensed. 4–5 hrs/wk.
FHIR Infrastructure	Pavel, CEO Health Samurai (Aidbox)	Free dev license, possible advisor equity. Activates at break-even.
Development Partner	Jacob Pielke, Cohesion Health	Foundation sprint + ongoing code review, architecture governance.

REVENUE THESIS

Layer 1 — Private Pay: $35/hr generates $2,600–7,500/month per family. HSA/FSA eligible. Break-even: 10 families at 20 hrs/week. Target: Month 3. Layer 2 — Medicare: PIN/CHI billing ($85–120/month per eligible client). ACCESS Model ($360–420/year per beneficiary). Activates Month 2. Layer 3 — Social Currency: Time Bank credits offset premium costs, extend care hours, and build community pipeline. GivePulse verification makes it auditable. Layer 4 — Insurance Data Moat: Continuous CareMCP biometrics + CII/CRI + daily care notes = richest aging-in-place health dataset in America. Powers AIP insurance underwriting no traditional agency can match. Projected: $5–10M underwriting capacity by Year 2.

3. What's Already Built

Four production-ready React components totaling approximately 4,800 lines. Built, tested, and functional. They need to be deployed on production infrastructure with HIPAA-compliant data storage and payment processing. That is the sprint scope.

Component	Lines	Description
CII Assessment	~800	12-dimension caregiver burnout tool. Green/Yellow/Red zones. Email capture + resource matching.
CRI Assessment	~1,000	14-factor clinical acuity. ADLs, cognition, medical complexity. Weighted scoring. Service tier matching.
Website v3 (National)	~950	National federation site. CareOS Health Intelligence dashboard. Wearable data flow. All CTAs functional. Both assessments embedded. HSA/FSA pricing.
Strategic Operating Brief	~1,100	Interactive execution dashboard with timeline, deliverables, and partnership status.

3.1 Website v3: Key Features

Feature	Status	Description
CareOS Health Intelligence	BUILT	Interactive dashboard: 5 live health signals from wearables with AI analysis and caregiver/MD notification status.
Wearable + MCP Integration	BUILT	Badges for Apple Watch, Oura, Garmin, WHOOP, Fitbit. MCP Protocol badge. 5-step data flow pipeline.
National Interest Gathering	BUILT	Start-a-Co-op form: location, role, community size, local story. Positions federation model.
All CTAs Functional	BUILT	Apply, Join Time Bank, Request Employer Pilot, Start a Co-op — all with validation + success states.
Embedded Assessments	BUILT	Full CII + CRI flows interactive within the website. No external navigation.
Money Flow Visualization	BUILT	$26 wages + $9 ops = $35 family pays. Transparent, zero extraction.

4. What I Need Built

Infrastructure requirements. Technology choices are yours. You know what's fastest to build, easiest to maintain, and best for our collaboration model.

4.1 Infrastructure Requirements

Layer	Requirement	Details
Hosting	SSL, CDN, custom domain	co-op.care resolves to production. SSL active.
Database	HIPAA-compliant, BAA	Encryption at rest, role-based access, real-time capable.
Auth	Multi-role access	Admin, caregiver, family, timebank_member roles. Email/password + OAuth.
Payments	Stripe MCC 8099	HSA/FSA direct. $100 deposits. Subscriptions. ACH.
Email	Transactional	Assessment alerts, payment receipts, Red Zone alerts, Time Bank confirmations.
Framework	Your call	SSR for SEO + SPA for assessments/CareMCP. AI-coding friendly.
MCP Server	CareMCP	Node.js + SSE transport. Wearable OAuth. Smart home connectors. See Sec 8.
Social APIs	GivePulse + Truemed	Verified volunteer hours. Automated LMN issuance. See Sec 9.

4.2 Data Model

Collection/Table	Key Fields	Access
users	email, displayName, role, status, socialCurrencyBalance	User: own. Admin: all.
assessments	type (cii|cri), userId, scores{}, totalScore, zone, reviewedBy	User: own. Admin: all. Josh: flagged.
families	primaryContact, patientInfo (encrypted), serviceTier, assignedCaregivers[], billingId, hsaAccountId	Family: own. Admin + CGs.
caregivers	userId, certifications, isFamilyMember, availability, hourlyRate, equityUnits	CG: own. Admin: all.
payments	familyId, amount, type, stripeSessionId, hsaUsed, accountType (hsa|lifestyle|social), receiptUrl	Family: own. Admin: all.
careNotes	caregiverId, familyId, date, content (encrypted), duration, gpsVerification, sdohCodes[]	CG writes. Admin + family reads.

Collection/Table	Key Fields	Access
healthSignals	patientId, source, type, value, timestamp, alertTriggered, caregiverNotified, mdReviewed	CareOS writes. CG + family reads. MD reviews.
wearableConnections	patientId, provider, oauthTokens (encrypted), lastSync, status	System R/W. Admin manage.
timeBankCredits	userId, hours, verifiedBy (givepulse), activityType, recipientId, creditValue, redeemed	User: own. Admin: all. GivePulse: write verified.
socialPrescriptions	patientId, referringProvider, serviceType, sdohCategory, referralNetwork (uniteus), status, closedLoop	Josh writes. CG + family reads. Unite Us: status updates.
lmnRecords	patientId, issuedBy (josh), condition, servicesApproved, truemedSessionId, validThrough	Josh writes. Family reads. Truemed: issue triggers.

4.3 HSA/FSA + Truemed Integration

Truemed automates Letter of Medical Necessity issuance. A family member completes a 2-minute telehealth evaluation. If they qualify (chronic pain, anxiety, mobility limitations, cognitive decline), Truemed issues an LMN that makes co-op.care services HSA/FSA eligible. Josh can also issue LMNs directly for complex cases.

Step	Actor	What Happens
1. Family requests care	Family	Completes CRI assessment. Platform identifies service tier and HSA/FSA eligibility.
2. LMN evaluation	Truemed API or Josh	2-min telehealth eval via Truemed, or Josh writes LMN directly for existing patients.
3. LMN issued	Truemed / Josh	Letter of Medical Necessity stored in lmnRecords. Linked to family's billing profile.
4. HSA/FSA payment	Stripe MCC 8099	Family pays with HSA/FSA card. Stripe processes at MCC 8099. Receipt includes reimbursement language.
5. Ongoing validation	Josh (quarterly)	Reviews active LMNs quarterly. Renews or adjusts based on CRI reassessment and CareMCP data.

5. The Foundation Sprint

One focused week. ~40 hours. Production infrastructure + developer foundation. Everything needed for launch and for me to start building CareMCP and API integrations under your review.

WHY 1 WEEK IS ENOUGH

4,800 lines of production React already built. You're not building from scratch. The scope is: (1) HIPAA infrastructure, (2) deploy existing components, (3) Stripe + HSA/FSA, (4) minimal admin, (5) developer foundation with CareMCP + API integration points.

FOUNDATION SPRINT (1 Week)

Task	Hours	Priority	Acceptance Criteria
HIPAA-compliant infrastructure	3	CRITICAL	BAA executed. Non-compliant services disabled. Config exported for local dev.
Database security (multi-role RBAC)	4	CRITICAL	Admin/caregiver/family/timebank roles enforced at DB level. Audit logging on PHI.
Deploy CII + CRI assessments	8	CRITICAL	co-op.care/assess/cii and /cri. AES-256 encryption. Red Zone → email to admin + Josh.
Deploy website v3 (national)	3	CRITICAL	co-op.care serves full site with CareOS, all CTAs, embedded assessments. SSL.
Stripe + MCC 8099 + HSA/FSA	6	CRITICAL	MCC 8099. $100 deposit + subscription. HSA/FSA + ACH. Receipts with reimbursement.
Admin dashboard (minimal)	6	HIGH	View assessments, payments, form submissions (apply/timebank/employer/coop). CSV export.
Email notifications	2	HIGH	Assessment → admin. Red Zone → Josh. Payment → family. Form → admin.
Git + CI/CD + foundation	4	CRITICAL	Protected main. Dev branch. CI: lint + type + test. PR template. CareMCP + API integration points documented.
Architecture handoff document	4	CRITICAL	For AI consumption. Structure, schema, security, Stripe, deploy, patterns. Includes CareMCP scaffold, GivePulse/Truemed/Unite Us integration architecture.

ACCEPTANCE TEST

Sprint complete when: 1. Family visits co-op.care on mobile. 2. Sees CareOS dashboard. 3. Completes CII + CRI. 4. Red Zone triggers email. 5. Pays $100 deposit with HSA debit card. 6. Receipt includes reimbursement language. 7. Admin shows everything. 8. All data encrypted under BAA. 9. I can clone, run, push, PR. 10. Architecture doc includes CareMCP + API ecosystem

integration points.

6. How We Work Together After the Sprint

I join your team. You don't join mine.

After the sprint, I build features — CareMCP, Truemed integration, GivePulse Time Bank, Unite Us referrals — under Cohesion's technical guidance. Your team reviews weekly, fixes critical issues, teaches me. No code reaches production without your approval.

6.1 Weekly Workflow

Day	What I Do	What Cohesion Does
Mon–Wed	Build features from roadmap using AI tools + architecture doc. Test locally.	Async Slack. No active time required.
Thursday	Push to dev. Open PR.	Review: SHIP IT / FIX THESE / REDO THIS.
Friday	Address comments. Push fixes.	Re-review. Approve. I deploy.

6.2 The Flywheel

HOW THIS GETS BETTER OVER TIME

Week 1: Heavy review. Every comment is a lesson. Week 4: Prompts are better. Patterns absorbed. Review lighter. Week 8: Code mostly ships clean. CareMCP v1 live. GivePulse integrated. Week 12: Routine features independent. Your role shifts to architecture decisions. Truemed + Unite Us live. AIP insurance data flowing. Meanwhile: Cohesion introductions to healthcare orgs, co-created content, and a case study covering regulated healthcare + MCP + social impact APIs.

7. The Deal

7.1 Cash

Payment	Amount	Timing
Sprint start	$2,500	On signing
Sprint delivery	$2,500	When acceptance test passes
Total	$5,000	Within 2–3 weeks

7.2 Equity

Option	Vehicle	Value	Details
A (Recommended)	SAFE Note	$15,000	Converts at $1M cap or next funding event. No immediate tax.
B	Restricted Units	$15,000	Direct units. 12-mo cliff, 36-mo vest. K-1.
C	Revenue Share	2% gross to $30K	Simple. Caps upside.

7.3 Ongoing Collaboration

Period	Cohesion Provides	I Pay	I Provide
Months 1–3	Weekly code review. Critical fixes. Async Slack. CareMCP + API architecture.	$0 (equity)	Healthcare SME. LinkedIn. Case study.
Month 4+	Same + expanded API review.	$1–2.5K/mo retainer	BD support. Network intros. Conference visibility.

THE MATH

Sprint: 40 hrs for $5K cash + $15K equity. Months 1–3: ~25 hrs code review covered by equity. Total: 65 hrs for $20K ($308/hr). Plus: healthcare domain expertise, 20K LinkedIn, conference intros, referral pipeline, and a case study covering regulated healthcare + MCP + GivePulse + Truemed + Unite Us + SDOH — the most complete social impact health tech stack any development shop has built. After Month 3, co-op.care is cash-positive. Cohesion has a recurring client, a healthcare + MCP + social impact showcase, and an active BD partner.

8. CareMCP Server + IoT Health Intelligence

The data moat traditional agencies can't build.

CareMCP is co-op.care's own Model Context Protocol server. It connects wearable health data and smart home IoT sensors from aging adults directly into CareOS clinical workflows. AI detects patterns, caregivers get alerts, physicians get clinical-grade data, and families get peace of mind. This transforms the CRI assessment from a one-time snapshot into a living, continuously-updated health picture.

8.1 What Is MCP?

The Model Context Protocol is an open standard (developed by Anthropic) for connecting AI systems to external data sources. Nori.ai has demonstrated this with their HealthMCP server for consumer wearable data. CareMCP goes further: it connects wearable data to a clinical care platform with physician oversight, caregiver alerts, and FHIR-compliant documentation.

The difference: "check my sleep score" vs. "detect that mom's sleep pattern correlates with UTI onset and alert her caregiver before she falls."

8.2 Architecture

Component	Technology	Function
MCP Server	Node.js + SSE transport	Serves health data to CareOS AI via MCP. Exposes tools for signals, trends, alerts.
OAuth Connectors	Per-provider OAuth 2.0	Authenticates with each wearable/IoT platform. Encrypted tokens. Auto-refresh.
Data Normalization	Custom mapping engine	Transforms vendor-specific formats into standardized CareOS health signals.
Signal Processing	Time-series analysis	Rolling averages, trend detection, anomaly scoring against personal baselines.
Alert Engine	Event-driven	Threshold crossed: (1) log, (2) notify caregiver, (3) flag for MD if clinical.
FHIR Bridge	Aidbox (Phase 2)	Maps normalized signals to FHIR R4 Observations. Clinical-grade documentation.

8.3 Data Sources: Wearables + Smart Home

Source	Key Signals	API / Protocol	Phase
Apple Watch	Heart rate, HRV, sleep,	HealthKit / Health export	1

Source	Key Signals	API / Protocol	Phase
	steps, fall detection, SpO2		
Oura Ring	Sleep quality, HRV, readiness, temp deviation, SpO2	Oura API v2 (OAuth 2.0)	1
Garmin	Heart rate, steps, sleep, stress score, body battery, fall detection	Garmin Health API (OAuth 1.0a)	2
WHOOP	Strain, recovery, sleep, HRV, respiratory rate, skin temp	WHOOP API (OAuth 2.0)	2
Fitbit	Heart rate, sleep stages, SpO2, steps, stress mgmt	Fitbit Web API / Google Health Connect	3
Smart Pill Dispenser	Medication adherence, dose timing, missed doses	Vendor API or Z-Wave hub	3
Motion Sensors	Room movement, bathroom frequency, nighttime activity	Z-Wave / Zigbee via hub	3
Bed Sensors	Sleep position, restlessness, bed exit (fall risk)	Vendor API or MQTT	3
Door Sensors	Departure times, unexpected exits, wandering detection	Z-Wave / Zigbee via hub	3

8.4 Health Signals + Clinical Thresholds

Signal	Warning	Alert	Clinical Action
Sleep Quality	<6.5 hrs or <75% eff	<5 hrs or <60% eff	MD reviews meds. CG adjusts evening routine.
Fall Risk	1 balance irregularity	2+ events or 1 fall	Auto-escalate to Josh. Home safety reassessment.
HRV	Drop >15% from 14-day avg	Drop >25% or <25ms	CG monitors. MD reviews if sustained 3+ days.
Activity	Drop >25% from baseline	Drop >50% or <1K steps	CG adds activity plan. Time Bank for walks.
Medication	85–95% adherence	<85% adherence	CG follows up within 30 min. MD reviews regimen.
Nighttime Motion (IoT)	>3 bathroom trips/night	>5 trips or unusual pattern	Screen for UTI/prostate. Adjust fluid timing.
Wandering (IoT)	Door open after 9pm	Unexpected departure	Immediate CG + family alert. GPS if available.

8.5 MCP Server Tools

MCP Tool	Parameters	Returns
get_health_summary	patientId, timeRange	Aggregated signals across all devices. Current status per type.
get_signal_trend	patientId, signalType, days	Time-series data. Rolling avg, trend, anomaly score.
get_active_alerts	patientId (optional)	Unresolved alerts. Severity, signal data, recommended action.
get_wearable_status	patientId	Connected devices, last sync, data freshness, issues.
acknowledge_alert	alertId, caregiverId, action	Marks addressed. Logs response. Updates timeline.
get_care_context	patientId	CRI score + wearable signals + care notes + alerts. Full picture.
get_iot_status	patientId	Smart home sensor status, recent events, anomalies.

THE COMPETITIVE MOAT

Traditional agencies have: a scheduling app and a billing system. No biometric data, no continuous monitoring, no AI, no clinical documentation. co-op.care with CareMCP has: continuous wearable + smart home monitoring, AI anomaly detection against personal baselines, real-time caregiver alerts, physician-reviewed clinical documentation, FHIR-compliant records, and GivePulse-verified community care labor. This is the richest aging-in-place dataset in America. It powers AIP insurance underwriting. It powers predictive care. It powers Medicare outcomes. No traditional agency can build this — private equity owns their data relationship. Our cooperative members own theirs.

WHAT COHESION GETS

CareMCP positions Cohesion at the intersection of MCP + healthcare + IoT + social impact. A production HIPAA-compliant MCP server connecting wearables and smart home sensors to clinical care with physician oversight. Every hospital system, health plan, and digital health company will want this within 2 years. Cohesion will already have built it.

9. Care Commons API Ecosystem

The middleware between community care and clinical outcomes.

co-op.care doesn't operate in isolation. It sits at the center of an API ecosystem that connects wellness commerce, volunteer management, clinical referral networks, and financial processing into a single care coordination platform.

9.1 API Integration Map

Integration	API	What It Does for co-op.care	Build Phase
Truemed	Truemed Partner API	Automates Letter of Medical Necessity issuance. 2-min telehealth eval → LMN → HSA/FSA eligible. Saves families 28–36%. Frictionless tax benefit.	Phase 1 (Wk 6)
GivePulse	GivePulse RESTful API	Verifies volunteer hours for Time Bank. City of Boulder + CU Boulder's official platform (70K+ users). Provides proof-of-service for social currency minting.	Phase 1 (Wk 8)
Unite Us	Unite Us Referral API	Closed-loop care referrals. Josh prescribes a home modification or community service. Unite Us tracks fulfillment. Payment/credit released only when service verified.	Phase 2 (Wk 10)
PIPs Rewards	PIPs Agile API	Gamified social currency wallet. Track behaviors (volunteer hours, wellness activities, transit usage). Mint digital credits. Redeem for premium offsets or co-op services.	Phase 2 (Wk 12)
Mindbody	Mindbody API v6	Wellness merchant integration. Members book yoga, fitness, nutrition coaching through cooperative ecosystem. Lifestyle Account payments processed via cooperative.	Phase 3
Flex	Flex HSA/FSA API	Alternative to Truemed for LMN-less HSA/FSA processing at partner merchants. Embeds pre-tax payment at checkout for qualifying wellness services.	Phase 3
ROOK / Thryve	ROOK Connect API	Unified wearable data aggregation layer. Alternative to per-vendor OAuth. Single API for Apple Health, Fitbit, Garmin, Samsung, WHOOP.	Phase 2 alt

9.2 Social Currency System Architecture

The Time Bank is co-op.care's social currency engine. It converts verified community care labor into economic value within the cooperative ecosystem.

Step	System	What Happens
1. Member volunteers	co-op.care platform	Neighbor provides companionship, transport, meal prep, yard work, tech help to aging member.
2. Hours verified	GivePulse API	Volunteer logs hours in GivePulse (City of Boulder official platform). GivePulse verifies via check-in/check-out, supervisor confirmation, or GPS.
3. Credits minted	co-op.care + PIPs	Verified hours → social currency credits at $15/hr base rate. Credits appear in member's digital wallet.
4. Credits redeemed	co-op.care platform	Member redeems credits for: (a) premium offsets on care services, (b) Lifestyle Account wellness purchases, (c) transfer to another member, (d) donate to cooperative community fund.
5. Impact tracked	GivePulse + Aidbox	Hours roll up into community impact metrics. Reported to City, CU Boulder, grant funders. FHIR: mapped to SDOH Observations.

9.3 Closed-Loop Referral Network

When Josh identifies a social determinant of health need — home modification, nutrition support, transportation, social isolation — co-op.care creates a "social prescription" routed through the Unite Us referral network.

Step	Actor	What Happens
1. Need identified	Josh / CareOS AI	CRI assessment or CareMCP signals reveal SDOH need (e.g., fall risk → grab bars needed).
2. Social prescription	Josh via platform	Creates referral in socialPrescriptions table. Specifies SDOH category (Gravity Project coded).
3. Referral sent	Unite Us API	Referral transmitted to appropriate community organization in Unite Us network.
4. Service delivered	Community org	Grab bars installed, meals delivered, transport arranged — whatever was prescribed.
5. Loop closed	Unite Us API	Service fulfillment confirmed. Status updated. Payment/credit released.
6. Outcome measured	CareOS + Aidbox	Follow-up CRI or CareMCP data confirms improvement (e.g., fall risk decreased). FHIR documented.

THE GRAVITY PROJECT + FHIR SDOH

The Gravity Project (HL7 FHIR) provides standardized codes for Social Determinants of Health. co-op.care maps every social prescription, Time Bank interaction, and community referral to Gravity Project codes. This means: 1. Josh's social prescriptions are interoperable with any FHIR-connected health system. 2. CareMCP health signals + SDOH referrals create a complete picture: clinical + social + behavioral. 3. CMS ACCESS Model reporting includes SDOH impact data — differentiating co-op.care from every other applicant. 4. Insurance underwriting incorporates social support networks, not just clinical data. This is the Care Commons made technically real: clinical care, community labor, and social determinants connected through standardized APIs.

10. Phase 2: Aidbox FHIR + SDOH Architecture

When co-op.care hits break-even, the architecture adds Aidbox for FHIR-native storage. Now with three data streams: operational (users, payments), clinical (CareMCP health signals, physician notes), and social (Time Bank credits, SDOH referrals, community impact).

10.1 Triple-Layer Architecture

Layer	Tech	Data	Pattern
Operational	Phase 1 platform	Users, assessments, payments, scheduling	Real-time, user-facing
Clinical	Aidbox (FHIR R4)	Patient records, Observations, Conditions, CarePlans	Medicare billing, hospital interop
Health Intelligence	CareMCP Server	Wearable + IoT signals, trends, alerts, anomalies	Continuous monitoring, AI analysis
Social	GivePulse + Unite Us + PIPs	Verified volunteer hours, SDOH referrals, social currency	Community impact, SDOH tracking
Translation	Serverless functions	Maps all layers → FHIR R4 with Gravity Project SDOH codes	Event-driven sync

10.2 CareMCP + Social → FHIR R4 Mapping

Data Source	FHIR R4 Resource	Code System	Example
CareMCP: sleep quality	Observation	LOINC 93832-4	Sleep 6.2 hrs, 67% efficiency
CareMCP: fall risk	Observation + Flag	SNOMED 129839007	Balance irregularity via Garmin
CareMCP: HRV	Observation	LOINC 80404-7	HRV 34ms, trending down
CareMCP: IoT motion	Observation	Custom + LOINC	5 bathroom trips overnight
Time Bank: volunteer hours	Observation (SDOH)	Gravity: social-connection	4 hrs companionship this week
Social Rx: home mod	ServiceRequest + Task	Gravity: housing-instability	Grab bars prescribed → installed
Social Rx: transport	ServiceRequest + Task	Gravity: transportation-insecurity	Medical transport arranged via Unite Us
LMN: medical necessity	DocumentReference	IRS Pub 502 / ICD-10	Josh: in-home care medically necessary
Clinical decision	ClinicalImpression	SNOMED procedure	Adjusted meds based on CareMCP

Data Source	FHIR R4 Resource	Code System	Example
		codes	sleep data

10.3 PIN/CHI Billing

Code	HCPCS	Description	Rate
CHI Initial	G0019	First 60 min community health integration/month	~$85
CHI Additional	G0022	Each additional 30 min/month	~$40
PIN Initial	G0023	First 60 min patient navigation/month	~$85
PIN Additional	G0024	Each additional 30 min/month	~$40

11. Feature Roadmap + 12-Week Timeline

11.1 Post-Sprint Features (My Builds, Your Reviews)

#	Feature	When	Revenue / Impact	Review Focus
1	CareOS voice note pipeline	Wk 4–6	Enables PIN/CHI	PHI encryption, error handling
2	CareMCP v1: Apple Watch + Oura	Wk 4–7	CareOS differentiator	MCP architecture, OAuth, HIPAA
3	PIN/CHI billing generator	Wk 5–7	$85–120/mo per client	Billing accuracy, time calcs
4	Truemed LMN integration	Wk 6–7	HSA/FSA automation	API security, LMN storage, Stripe flow
5	Family dashboard + signals	Wk 6–8	Retention + referrals	Auth, real-time data, no PHI leakage
6	GivePulse Time Bank integration	Wk 7–9	Social currency launch	Hour verification, credit minting logic
7	Caregiver mobile PWA + alerts	Wk 7–9	Ops efficiency	GPS, push notifications, CareMCP alerts
8	CareMCP v2: Garmin + WHOOP + IoT	Wk 8–10	Broader monitoring	Additional OAuth, data normalization
9	Unite Us referral integration	Wk 9–11	Closed-loop SDOH	Referral API, status tracking, loop closure
10	BCH discharge integration	Wk 8–10	$2.5–5K/mo retainer	HL7 parsing, webhook security
11	Aidbox FHIR + CareMCP Bridge	Wk 10–14	ACCESS eligibility	FHIR R4, Gravity Project SDOH mapping
12	PIPs social currency wallet	Wk 12–14	Gamified engagement	Credit logic, redemption, abuse prevention
13	AIP insurance data export	Wk 14	Underwriting moat	Anonymization, actuarial format

11.2 Week-by-Week Timeline

Week	Dates	Owner	Milestone
1	Mar 3–7	Blaine	CDPHE license. Josh retainer. CMS-855B. First 5 families.

Week	Dates	Owner	Milestone
2	Mar 10–14	Cohesion	SPRINT: infrastructure + deploy + Stripe + admin + architecture doc.
3	Mar 17–21	Blaine	First caregiver. FIRST FAMILY. Revenue starts. First PR.
4	Mar 24–28	Blaine + review	CareMCP scaffold. Apple Watch OAuth research. Voice pipeline.
5	Mar 31–Apr 4	Blaine + review	5–6 families. CareMCP v1 testing. Truemed integration started.
6–8	Apr 7–25	Blaine + Josh	10 families. CareMCP live. Truemed live. GivePulse integrated. PIN/CHI generating. Break-even.
9–12	Apr 28–May 23	Blaine	15 families. Unite Us referrals. CareMCP v2. AIP data flowing. BCH formalized. Cash positive.

THE LONG VIEW

Month 4 (June): Cash-positive. CareMCP live with 2–3 wearable providers. Truemed automating LMNs. Time Bank active. Retainer negotiation. Month 6 (August): 20–25 families. Aidbox activation. CareMCP + SDOH data flowing into FHIR. Unite Us referrals closing loops. AIP insurance model in testing. Month 8 (September): ACCESS Model application with CareMCP + SDOH data no other applicant has. GivePulse-verified community impact metrics. Month 12 (January 2027): Second cooperative launching. AIP insurance underwriting from the richest aging-in-place dataset in America. CareMCP expanding to childcare safety monitoring. Cohesion has: healthcare vertical, MCP reference implementation, social impact API showcase, and a 20K-connection LinkedIn advocate. The sprint builds the foundation. CareMCP builds the health intelligence. The API ecosystem builds the Care Commons. The partnership builds Cohesion's healthcare practice.

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